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Correct
Answer

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Practice Exam - 1

Question 96 of 117



A 56-year-old woman with a history of rheumatoid arthritis presents for evaluation of new onset heart failure. She has a 20-year history of arthritis which is well controlled on daily ibuprofen and hydroxychloroquine as well as intermittent steroids. She has NYHA class III symptoms for the last 2 months. Her ECG is notable for a 1st degree AV block and right bundle branch block.

A still image of her echocardiogram is shown below.



Parasternal Short Axis Image of Patient

Which evaluation is most likely to identify a complication of HCQ?

A. Genetic Testing



B. Cardiac MRI

C. Echocardiography



D. Endomyocardial biopsy

E. No further testing is required

Commentary

References

Testing Point: The Board-certified AHFTC specialist should be able to recognize chloroquine toxicity.

Correct Answer: D. Endomyocardial biopsy.

Rationale: Cardiac *Biopsy*. Chloroquine toxicity is a rare but severe side effect of sustained use of hydroxychloroquine. It classically presents with heart block and, in its most severe form, heart failure. Biopsy is indicated to make the diagnosis. Electron microscopy of biopsy tissue is notable for numerous autophagic vacuoles and curvilinear bodies (see Figure 2).

Answer A: Genetic testing, though could be useful, would not be diagnostic in this scenario.

Answer B: Though an MRI could be useful, findings are not specific for the diagnosis of chloroquine toxicity.

Answer C: Echocardiography has no findings specific for chloroquine toxicity and is not a definitive test. Because withdrawal of chloroquine therapy may be necessary, the diagnosis should be confirmed by biopsy prior to changing therapy for rheumatoid arthritis in a patient whose disease is controlled and biopsy should be performed, thus answer E is incorrect.

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